Hazard Identification and Inspection Protocol Sheet

Long-term Care (CU 766011)

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Long-term Care (CU 766011)

Industry or Classification Unit Information

Name of Industry or Classification Unit	Long-term Care (Classification Unit #76011)
Description of typical work conducted in this Industry or Classification Unit	This classification unit covers firms that operate medical facilities providing residential care services to individuals that have complex and extended health care needs. **Extended Care:** This is health care provided to persons having a severe chronic disability which has usually produced a functional deficit which requires 24-hour-a-day professional nursing services and continuing medical supervision, but does not require all the resources for an acute care hospital. Most persons at this level have a limited potential for rehabilitation and often require institutional care on a permanent basis. **Extended Care Hospital:** A hospital that primarily exists for the continuing treatment of patients who have a long term illness or have a low potential for recovery, and who require regular medical assessment and continuing nursing care. **Intermediate Care:** There are three levels of intermediate care. Level 1 is for persons who are independently mobile with or without mechanical aids, require moderate assistance with the activities of daily living, but require daily professional care and/or supervision. Level 2 is the same as level 1, except for the need for heavier care and/or supervision requiring additional care time. Level 3 is for psychogeriatric persons who have severe behavioural problems on a continuing basis. Persons requiring a heavier level of care involving considerably more staff time than level 2, but who are not eligible for extended care, may also use level 3.

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	Residential Care Facility: Includes homes for the aged, physically handicapped, mentally handicapped, developmentally delayed, psychiatrically disabled, and clients with alcohol and drug problems. Although there is some overlap with extended care hospitals, these facilities generally do not provide the same level of care. Many facilities are maintained for people chronically ill or disabled who live there more or less permanently. Other facilities provide shelter for a shorter period of time and often provide a program or service. **Specialized Health Care:** The provision of board and lodging to an adult person, where the major disabilities are mental health or alcohol or drug related and may include supervision, personal or intermediate care, and social or educational training. **Personal Care:** Health care provided to persons who are independently mobile with or without mechanical aids to assist them with the performance of personal functions and activities necessary for daily living. Examples of facilities covered by this classification unit are as follows: • Continuing care facility • Extended care facility • Long term care facility • Nursing home

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Industry or Classification Unit Hazards

Does this industry or classification unit contain hazards relating to **confined space entry**?	Few tasks are associated with confined space entry hazards. Categories of workers who may be exposed to hazards are as follows: **Building operations/maintenance:** • Providing maintenance for a sump pump, compactor, dryer
Does this industry or classification unit contain hazards relating to **working from heights**?	Normal work tasks associated with maintaining a building may be present. Categories of workers who may be exposed to hazards are as follows: **Building operations/maintenance:** • Changing light bulbs. • Interior maintenance or outside work (i.e., cleaning eves, working on the roof). **Support services:** • Cleaning at height (i.e., using a step ladder).
Does this industry or classification unit contain hazards relating to **machine safeguarding**?	Few tasks are associated with machine safeguarding hazards. Categories of workers who may be exposed to hazards are as follows: **Care staff:** • Few tasks (i.e., using a sterilizer/hopper). **Building operations/maintenance:** • Basic carpentry equipment (i.e., table saw, band saw, power tools, lawn mower). **Support services:** • Washer/dryer. • Mixer, blender. • Meat slicer.

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	• Other kitchen equipment.
Does this industry or classification unit contain hazards relating to **machine and equipment lockout**?	Few tasks contain this hazard. As above, with minimal expectation that care staff and support services would be required to perform a task that required lockout.
Does this industry or classification unit contain hazards requiring **respiratory programs**?	There are hazards that require respiratory programs Type of hazards – Biological hazards (i.e., H1N1, SARS, influenza, TB, “superbug”) and air contaminant (i.e., asbestos, lead, controlled products used at the worksite) Workers may be exposed to hazards when: Categories of workers who may be exposed to hazards are as follows: **Care staff:** • Providing direct care to an infected resident (i.e., personal care, bathing, dressing, feeding, administrating medication, respiratory therapy). **Building operations/maintenance:** • Performing a task in the room of an infected resident. • Performing work that may expose them to air contaminants such as asbestos or lead. **Support services:** • Performing a task (i.e., housekeeping, changing linen) in the room of an infected client and using a controlled product (i.e., cleaner).

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Does this industry or classification unit contain hazards relating to **violence in the workplace?**	Risk level varies depending on providing direct or indirect care and interaction with sources of violence (i.e., residents, family members, friends, visitors, etc.) Categories of workers who may be exposed to hazards are as follows: **Care staff:** • Providing direct and indirect care to clients. • Interaction with family/visitors. • Transporting residents in vehicle. • Accompanying resident into community. • Working alone (i.e., on evening or night shifts). • Working directly with people who have a history of violence, abuse drugs or alcohol, gang members, and relatives of patients or clients. • Working in a special care unit where clients have free movement and may be unpredictable. • One-on-one assignments without knowledge. **Building operations/maintenance:** • Interacting with family/visitors/residents. • Working alone. **Support services:** • Interacting with family/visitors/residents. • Working alone. Factors involved in a violence incident are as follows: • Access to items that could be used as a weapon (i.e., entry into nursing station and using item on desk or into med room). Client cognitive ability – concurrent disorders, having impaired judgment, use of illegal drugs or alcohol on the worksite or coming from the community under the influence of drugs/alcohol, cognitive decrease, neighborhood situation, changes but not recognized or addressed in documentation or upon entry to worksite. • Work environment – design and layout of rooms creates blind spots or single access/egress area.

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	• Layout of worksite and where it is situated (type of neighborhood). • Poorly lit corridors, rooms and external areas. • Lack of emergency communication, security system/response, use of personal alarms
Does this industry or classification unit require **Exposure Control Plans (ECPs**)?	ECPs would be based on exposure to biological agents (i.e., Norwalk, H1N1, SARs, influenza, blood and body fluids), and air contaminants (i.e., inhalation of hazardous substance - non ETS). Categories of workers who may be exposed to hazards are as follows: **Care staff:** • Exposed to biological agents and cytotoxic drugs while providing direct care. **Building operations/maintenance:** • Asbestos, lead, controlled products. **Support services:** • When cleaning biological/cytotoxic spills. • Laundry.
Does this industry or classification unit contain hazards of **musculoskeletal injuries (MSI)?**	Risk level varies depending on providing direct or indirect care and interaction with residents. Categories of workers who may be exposed to hazards are as follows: **Care staff:** • Overexertion – forceful exertion and awkward posture during client care (i.e., Lifting, repositioning and transferring resident). • Lack of safety devices (i.e., transfer board, slider sheet) or incorrect devices (i.e., incompatible sling for lift or task). • Walking an ambulatory client. • Lifting, reaching, or moving residents who cannot move on their own. • Awkward postures (i.e., pulling/lifting patients from tubs, whirlpools).

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	Support staff: • Laundry (i.e., Excessive reaching/pushing and/or lifting wet heavy laundry). • Housekeeping (i.e., pushing/pulling carts). **Building operations/maintenance:** • Performing normal tasks.
Does this industry or classification unit contain hazards of **working alone**?	Yes. Refer to the Violence in the Workplace section of this document.
Does this industry or classification unit contain hazards relating to **contractor safety**?	Risk level varies depending if contractors are part of the regular workforce at the site or if they are hired for specific projects. Categories of workers who may be exposed to hazards are as follows: **Care staff:** • Where contractor work occurs within the area that care staff are working (i.e., renovation of a resident's room or common area). • Contractors may also be care staff. **Support services:** • Contractors may include housekeeping, laundry and food services workers. **Building/facilities:** • Where contractor work occurs within the area that building/facilities staff are working. • Contractors may perform preventative maintenance, construction/renovation. • Contractors may include building/facilities workers.

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Does this industry or classification unit contain hazards relating to **blood and body fluid (BBF)/ biological agents/Other Potential Infectious Material (OPIM)**?	Risk level varies depending on providing direct or indirect care and interaction with residents. ECPs would be based on exposure to biological agents (i.e., Norwalk, H1NI, SARs, influenza, needle stick incidents, first aid). Refer to ECP section. Categories of workers who may be exposed to hazards are as follows: **Care staff:** • Providing direct / indirect care to residents (i.e., physical therapists provide therapy to patients with bed sores or other open wounds, nursing staff showering residents and may stand in water that may have blood or fecal matter). • Some facilities allow employees to rinse contaminated laundry (i.e., laundry contaminated with blood or Other Potentially Infectious Materials (OPIM) or that might contain sharps) in dirty utility "hopper" rooms instead of simply containerizing it and transporting it to the laundry. **Support services:** • Working indirectly/near infected resident. • Handling waste/garbage. • Laundry (i.e., use of lower water temp, less bleach). • Contaminated surfaces. • Cleaning up biological spills. **Building/facilities:** • Working indirectly/near infected resident. • Handling waste/garbage.

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Does this industry or classification unit contain hazards relating to **slips, trips, and falls**?	Normal work tasks associated with providing direct or indirect care may be present. Categories of workers who may be exposed to hazards are as follows: **Care staff:** • Providing direct care (i.e., moving resident from one surface to another surface, moving resident in bath chair to/from bathroom, working in crowded resident room). • Working in areas where there are transition surfaces, level changes, sources of liquid, cables and hoses, damaged flooring/paving and slippery surfaces. • Hallways crowded on both sides with furniture, equipment, etc. **Support services:** • Performing tasks in crowded work areas (i.e., laundry room, kitchen, resident's room). **Building/facilities:** • Performing tasks in crowded work areas.
Does this industry or classification unit contain hazards relating to **cytotoxins**?	Cytotoxin therapy is not a common therapy in this CU. The common method of administrating it is orally (by mouth). The byproduct of the therapy will be found in BBFs (i.e., urine and saliva). Categories of workers who may be exposed to hazards are as follows: **Care staff:** • Providing direct care and where there is soiled linen, spills of BBF on the floor. **Support services:** • Exposure to soiled linen. • Cleaning up spills. • Disposing of waste/garbage.

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Does this industry or classification unit contain other specific hazards not previously listed?	
Type of hazard	**Description of hazard**
Chemical agents	**Care Staff** • Care staff may be exposed to cleaners, sterilizing, and disinfecting agents (i.e., glutaraldehyde used to disinfect physical therapy whirlpools or tubs). • Staff may be exposed to mercury from accidental spills. **Support Services** • Housekeeping may be exposed through the use of cleaning, sterilizing and disinfecting agents. • Dietary staff may be exposed to ammonia, used as a cleaning agent, and chlorine solutions used as disinfectants in dishwashing; drain cleaners, oven cleaners, and grill cleaners can be caustic solutions that can cause skin burns and eye and skin irritations. • Staff may be exposed to mercury from accidental spills. **Facilities and Maintenance** • Facilities staff may be exposed to hazardous chemicals from cleaning and maintenance work. Including exposure to paints, adhesives, pesticides, solvents used in maintenance shops, waste anesthetic gases and ethylene oxide if repairing ventilation or exhaust systems that are used to remove these gases. • Staff may be exposed to mercury from accidental spills.
Latex	Any worker exposed to products that contain latex such as gloves, catheters, and tubing may develop a latex allergy.

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Prevention Officer Personal Safety While On-Site

Does this industry or classification unit require specific **Personal Protective Equipment (PPE)** the Prevention Officer must don while they are on-site?	Minimal PPE. If there is a reason to wear PPE other than in a 911/811 incident, then an inspection must not occur.
Does this industry or classification unit require the Prevention Officer to use **air monitoring equipment** while they are on-site?	No.
Does this industry or classification unit require the Prevention Officer to protect themselves against **dogs** while they are on-site?	There may be a dog on site. The Officer would remove themselves in incidents where they believe their safety is at risk.

Risk Assessment Process

Does this industry or classification unit require a specific **Risk Assessment Process** the Prevention Officer must follow while they are conducting an inspection?	The regular risk assessment process through research of the employer on WSM and WSE, and knowledge through research of hazards found in a health care worksite.

Personal Protective Equipment (for Workers)

Does this industry or classification unit require specific **Personal Protective Equipment** that the workers must don while they carry out their job functions?	PPE according to the site's ECPs and safe work procedures. Respiratory protection if unable to receive flu immunization.

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Regulations to Consider

Hazard/Violation/Topic	Regulation Reference
Violence	Occupational Health and Safety Regulation (OHSR) Sections 4.27-4.31, *Violence in the Workplace* • Specific references: s4.28, s4.29, s4.30
MSI	OHSR Sections 4.46-4.53, *Ergonomics (MSI) Requirements* • Specific references: s4.48, s4.50(1)
Working alone	OHSR Sections 4.20.1-4.23, *Working Alone or In Isolation* • Specific reference: s4.21
Biological agents	OHSR Sections 6.33-6.40, *Biological Agents* • Specific references: s6.34, s6.36
Slips, trips, and Falls	OHSR Sections 4.32-4.42, *Work Area Requirements* • Specific references: s4.39, s4.40
Cytotoxins	OHSR Sections 6.42-6.58, *Cytotoxic Drugs* • Specific references: s6.43, s6.48
Chemical agents (WHMIS)	OHSR Sections 5.3-5.18, *Workplace Hazardous Materials Information System (WHMIS)*
Controlling Exposure (to chemical and biological agents)	OHSR Sections 5.48-5.59, *Controlling Exposure*

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Additional Sources of Information

Topic	Source
OSHA Essentials for Healthcare	http://www.evolveelearning.com/course/osha-essentials-for-healthcare-training-courses.html
WorkSafeBC Industry and Labour Services Health Care SharePoint site	http://teamsites/sites/ts_ils2/Web%20Pages/Health%20Care.aspx
WSN (Health and wellness section)	http://wsn/SafetyHealthWellness/HealthWellness/Pages/default.aspx
Health and Safety at Work in Residential Care Facilities (Health and Safety Authority – Ireland)	https://www.hsa.ie/eng/Publications_and_Forms/Publications/Healthcare_Sector/Health_and_Safety_at_Work_In_Residential_Care_Facilities1.html

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